DELHI PRECISION MEDICINE CENTRE

Concept Design Brief

Qutub Institutional Area, Mehrauli, New Delhi 110016

April 2026 · Version 1.0 · Confidential · Architecture & Design Engagement

1. About Delhi Precision Medicine Centre

The Delhi Precision Medicine Centre (Delhi PMC) is a greenfield quaternary hospital being established by VPS Healthcare in the Qutub Institutional Area, Mehrauli, South Delhi. It is the Delhi node of a developing precision medicine network anchored at VPS Lakeshore Hospital, Kochi — one of India's foremost tertiary institutions with a 20-year track record in multi-organ transplantation, oncology, and complex care.

The Delhi PMC is purpose-built around five clinical programme verticals: Haemato-Oncology and BMT/CAR-T, Rare Disease and Immunology, Neuromuscular Medicine, Precision Oncology for Solid Tumours, and Organ-Specific Transplantation. Every programme in the building is precision-defined. This is not a general hospital with a speciality floor; it is a vertically integrated precision medicine institution from the ground up.

This brief commissions a full-scale concept design for the Delhi PMC — a G+6 building with five basement levels and roof, on a plot size of ~61 m × 67 m, delivering ~2,54,800 sq ft BUA and 106 licensed beds.

Promoter	VPS Healthcare — operator of VPS Lakeshore Hospital, Kochi and the Delhi Precision Medicine Centre network node
Location	Qutub Institutional Area, Mehrauli, South Delhi — PIN 110016. Shaheed Jeet Singh Marg / Sri Aurobindo Marg corridor. Approx. 28.526°N, 77.185°E.
Scale	~2,54,800 sq ft BUA · 106 Licensed Beds · 6 Operation Theatres G+ 6 + 5 Basements + Roof · Greenfield
Clinical focus	Haemato-Oncology & BMT / CAR-T · Rare Disease & Immunology · Neuromuscular · Precision Oncology — Solid Tumours · Organ-Specific Transplantation
Site context	IIT Delhi ~1 km north · JNU ~1.5 km west · Rockland Hospital and Sitaram Bhartia Institute in the same institutional zone · Metro (Yellow Line): Malviya Nagar ~1.2 km; Qutub Minar ~800 m south
Land use / zoning	Institutional Area per Delhi Master Plan (MPD-2041). FAR for institutional / hospital use: 3.5. Basements excluded from FAR. Plot coverage ceiling ~33–40%. Building plan approval: DUAC / MCD.

2. Facility Configuration & Programme Verticals

The Delhi PMC is a single-building institution. Unlike the Kochi campus — which operates as a two-block configuration — the Delhi PMC integrates all five programme verticals, associated diagnostics, and support infrastructure within one purpose-designed structure.

The five clinical programme verticals are defined below. The building's floor stack, room programme, and infrastructure specification all derive from the clinical logic of this vertical organisation.

#	VERTICAL	PROGRAMME CHARACTER
1	**Haemato-Oncology & BMT / CAR-T**	Long LOS · BMT + CAR-T · Continuous infusion · ICU-dependent · Highest revenue per episode
2	**Rare Disease / Metabolic / Immunology**	Lifelong patients · Predictable ERT / IVIG cycles · Low surgery, high daycare · Acute exacerbation planning · Genetics dependency
3	**Neuromuscular Programme**	Low volume, high complexity · Ventilator + ICU heavy · Gene therapy protocols · Respiratory crisis preparedness
4	**Precision Oncology — Solid Tumours**	Complex resections · High infusion · MDT-driven · Medium / unpredictable LOS · Higher complication rate
5	**Organ-Specific Transplantation**	Kidney (highest Tx volume) · Liver (high complexity) · Heart/Lung optional · Dedicated ICU and OT clusters

Procedure Spaces & Radiation

OT / PROCEDURE SPACE	CATEGORY	NOS / KEY NOTE
Transplant OTs	Specialty & Sterile	2 — liver/kidney; hepatobiliary; perfusion-capable
Oncology Surgery OTs	Specialty & Sterile	2 — complex resections; sarcoma; robotic-assisted; laminar flow
Hybrid OT	Specialty & Sterile	1 — IR + vascular + image-guided
Routine OTs	Specialty & Sterile	1 — day cases, port insertions, EUA
LINAC Vaults	Radiation Oncology	1 operational + 1 structural shell; commission second at ~40 Rx patients/day
HDR Brachytherapy	Radiation Oncology	1
CT Simulator	Radiation Oncology	1

3. Floor Configurations

The floor configurations should reflect a full clinical adjacency and infection control review. Key principles governing the stack: BMT positive-pressure and AIIR negative-pressure isolation must not share lift cores or uncontrolled corridors; immunocompromised patients must be vertically separated from infectious disease wards; OT suites must be above public-facing OPD floors; and Oncology Urgent Care must have a physically separate entry and waiting zone from elective OPD.

Bed & Space Count

106 licensed beds · 52 day care & infusion bays · 6 operation theatres · Radiation Oncology · Radiology · Dedicated parking · Stores · CSSD · Blood bank & Lab ·

Total Built Up Area - 254,800 Sq ft.

Space Count Summary

LICENSED INPATIENT BEDS	106	DAY CARE / TREATMENT BAYS	52
BMT Beds (HEPA+ pos. pressure)	10	Complex Infusion Bays (chemo, immuno, bispecifics)	18
Haemato-Oncology ICU	8	Rare Disease Infusion Bays (ERT, IVIG, factor)	10
Step-Down / HDU	8	Critical Infusion Beds (single cubicles)	6
AIIR Isolation	6	Apheresis (Blood Bank, B3)	4 chairs
ICU / Critical Care Subtotal (Licensed)	**32**	Dialysis Stations (transplant prep & bridging)	6-8
Haematology & Blood Disorders Ward	22	Clinical Trials Bays (Phase II/III)	8
Immunocompromised / High-Supervision Ward	10	**Operation Theatres**	**6**
Complex Care Ward	26	Transplant OTs (liver/kidney; perfusion-capable)	2
Palliative & Symptom Management	8	Oncology Surgery OTs (robotic; laminar flow)	2
Neuromuscular Zone (ventilator-capable)	4	Hybrid OT (IR + vascular + image-guided)	1
Ward Subtotal (Licensed)	70	Routine OTs (day cases, port insertions)	1
Pre & Post Operative	8-10	**Specialist Procedure Spaces**	
Endoscopy / DSA / RT Recovery	8	LINAC Vaults (Vault 1 live + Vault 2 shell)	2
Oncology Urgent Care	4	HDR Brachytherapy Suite	1
Ancillary Subtotal	**20-22**	CT Simulator (Radiation Oncology)	1
Phase I Clinical Trial Suites	4	BMT Procedure Room (laminar flow) & Brachytherapy Minor OT	2
TOTAL LICENSED BEDS	**106**	Endoscopy Suite	3
		DSA Lab	1
MDT Boardrooms (video-linked)	4		

4. Infrastructure Non-Negotiables

The following infrastructure elements are mandatory and must be specified in the design brief from concept stage. They cannot be retrofitted after structural decisions are made.

BMT Suite (F6)

- Dedicated AHU, HEPA H14 filtration, positive-pressure anteroom, dedicated lift — no shared elevator core with F5 AIIR floor
- Positive-pressure maintained at all room boundaries — anteroom to every BMT room
- Laminar flow in the Departmental Procedure Room
- Pharmacy satellite with ISO Class 5 cleanroom conditions for chemo and biologic preparation

AIIR Isolation (F5)

- Negative-pressure rooms with anteroom, independent exhaust to atmosphere, dedicated nursing station
- AHU completely separate from F6 BMT — no shared air handling across pressure zones
- Minimum 4 rooms; each room with its own anteroom and negative-pressure monitoring

OT Suite (F4)

- Minimum 2 Class 10,000 rooms with laminar flow, perfusion-capable, for transplant surgery
- Full sterile zone separation from F3 ward floor below — restricted-access staircase and lift core
- CSSD satellite on F4 for immediate OT turnaround; full CSSD in B2
- Hybrid OT: 1 room with biplane C-arm-capable design, lead-lining, image integration

PET-CT Uptake Rooms (B2)

- PET-CT suite with lead-lined doors and windows including the ancillary rooms as per norms,, 90-minute hold protocol, no shared waiting with general radiology patients
- Dedicated uptake corridor — no cross-flow with CT or MRI waiting areas

Pharmacy Compounding (B2)

- ISO Class 5 cleanroom for chemo and biologic preparation, cold-chain storage, IMP vault for clinical trials
- Narcotics vault: separate locked room, access-controlled

Radiation Oncology (Lower B4)

- LINAC vault 1: fully fitted and commissioned from Day 1
- LINAC vault 2: structural shell only — slab, shielding walls, MEP rough-in; fit-out deferred to ~40 Rx patients/day threshold
- Vault design: RCC 1.1m to 2.4 m primary shielding walls, roof slabs; maze entry; no direct line of sight to occupied areas
- Nuclear Medicine Hot Lab: independent ventilation exhaust, negative pressure, no recirculation

General — All Nursing Floors

- Dirty 11/ clean utility rooms, linen hold, and biomedical waste hold at every nursing floor
- Dedicated waste lift — no shared core with patient or visitor lift banks
- Family communication rooms at each ICU cluster

5. Strategic Context & Design Philosophy

This brief commissions a purpose-built precision medicine institution. The scope is accordingly singular: a greenfield building designed end-to-end for a defined clinical programme, a defined patient cohort, and a defined institutional identity.

The design must operate across three interlocking imperatives:

Clinical-First	Compliance-Driven	Experience-Led

Evidence-based adjacencies, flow logic, and clinical cluster design. Infection control embedded in architecture — not applied as an afterthought.	NBCI, NABH, JCI, fire safety, biomedical waste, MOEF EC, Delhi PCB, PwD norms — fully embedded from concept stage. Not a checklist; a design condition.	Dignified, calm, intuitive environments. Upscale but never ostentatious. The architectural language of competence, calm, and care.

The Five Design Imperatives

Every design decision must pass through the lens of these five principles:

- Flow — Zero confusion, zero delay. Legible circulation, separated movement streams, and intuitive spatial sequencing at every level. The rare disease cohort includes patients with mobility limitations, fatigue, and cognitive variation — the building must remove navigation load entirely.
- Time — Faster diagnosis, shorter stays. Parallel clinical workflows enabled by adjacency planning and diagnostic clustering. For transplant and BMT patients, time is a clinical variable.
- Continuity — Before, during, and after care: a seamless spatial and service continuum, from genomic prevention through to post-discharge. The Precision Prevention Clinic at Ground, the OPD stack at F1, the infusion and trials floor at F2, and the ward floors above are a single care pathway expressed architecturally.
- Dignity — Privacy, calm, and respect as spatial conditions — embedded in layout, materials, acoustics, and light. The patient cohort includes patients with rare, often stigmatised conditions, long treatment episodes, and high emotional load on families.
- Distinction — Differentiation through design intelligence: operational, spatial, and experiential. The Delhi PMC must read as a precision institution — not a conventional hospital with a genomics floor.

Design Tone — Upscale. The architectural language of this facility is to communicate competence, calm, and care. Materials, lighting, acoustics, and wayfinding are to collectively produce an environment that is oriented, respectful, and safe. The Precision Prevention Clinic at Ground sets the aesthetic register for the building: it is designed for well to do patients making a considered health investment — its atmosphere is aspirational, clinical-intelligence-led, and wholly distinct from a conventional hospital waiting area.

6. Comprehensive Scope of Works

01 Regulatory Compliance & Code Alignment

All design and construction output to conform to current national and state regulatory standards across applicable codes and accreditation frameworks.

- Review and ensure compliance: National Building Code of India (NBCI), Delhi Fire Safety Norms, NABH, JCI, Biomedical Waste Management Rules, MOEF EC, Delhi PCB, PwD Norms, Delhi Master Plan (MPD-2041) Institutional Area requirements
- Corridor width corrections — patient, bed, and fire evacuation compliant throughout
- Fire exit planning: adequate number and distribution, pressurised staircases, fire-rated doors and compartmentalisation across all floors
- Ceiling heights as per NBCI norms; hospital minimum 3.0 m clear height in clinical areas
- Smoke zoning and fire compartmentalisation
- Refuge areas and evacuation planning — including basement levels (Lower B4 through B4)
- Accessibility compliance: barrier-free design (ramps, lifts, toilets), PwD norms throughout; specific attention to the rare disease and neuromuscular patient cohort
- HVAC zoning for infection control — OTs, ICUs, BMT suite, AIIR isolation, and immunocompromised wards

02 Clinical Planning & Functional Allocation

Departmental organisation to be structured around clinical adjacency logic, infection control requirements, and patient flow. The floor configurations in Section 3 provide the design team's working brief; the architect is expected to develop and refine these.

- Departmental organisation based on clinical adjacency, patient flow logic, and essential area requirements for waiting, OPD rooms, and bed areas
- Segregation of flows: Patient / Attendant / Staff / Clean Material / Biomedical Waste — throughout
- Vertical and horizontal circulation optimisation: dedicated patient lifts, service lifts, emergency lifts, BMT-dedicated lift (F6)
- Oncology Urgent Care repositioning: dedicated entry, separate triage from elective OPD, no shared waiting
- ICU, OT, and diagnostics clustering for critical care efficiency
- Sterile vs non-sterile zoning — explicit delineation at every floor
- Pressure zone mapping: positive-pressure BMT (F6), negative-pressure AIIR (F5), negative-pressure Transplant ICU — no uncontrolled transitions between zones

03 Floor-Wise Accessibility & Circulation Planning

- Intuitive navigation across all floors: clear entry point at Ground, separated circulation for clinical and non-clinical staff
- Elimination of dead corridors and bottlenecks — especially critical on the ward floors (F3) and the OT floor (F4)
- Spine corridors for major patient and material movement
- Logical vertical stacking as defined in Section 3
- Wayfinding strategy: signage, visual cues, programme-specific colour-coded zoning per clinical vertical
- Accessibility for wheelchairs, stretchers, and emergency movement throughout — specific attention to basement-level radiation oncology patient flow (Lower B4)

04 Departmental Design & Standardisation

- Standardisation of room sizes and clinical layouts — OPD rooms, infusion bays, ICU bays, inpatient rooms — per NABH and JCI requirements
- Design of OPD cluster (F1) — 30 rooms, programme-differentiated waiting, nurse stations per NABH requirements
- Optimisation of nursing stations and support areas at every floor
- BMT suite room design to HEPA H14 standard with anteroom, positive-pressure monitoring, and sealed surfaces

05 Consolidation of Diagnostic & Support Services

- Diagnostics consolidated in basement levels: Radiology + CSSD + Pharmacy (B2); Laboratory + Genomics + Biobank (B3); Radiation Oncology + Nuclear Medicine (Lower B4)
- Basement consolidation frees clinical floors entirely for programme-specific ward, ICU, OT, infusion, and OPD use
- CSSD: single facility (B2) serving all OTs (F4) and BMT laminar flow room (F6) via dedicated clean supply lift
- Pharmacy: central cold-chain facility at B2 with satellites at F2 (trials / infusion) and F6 (BMT)

06 Institute and Programme Zones

Each programme zone to be established as a distinct, integrated zone with its own clinical identity, care pathway, and supporting infrastructure.

- Institute of Haemato-Oncology & BMT / CAR-T (F5–F6)
- Organ Transplant Programme — Liver, Kidney, Multi-organ (F4 OT; F5 ICU; F3 Ward)

- Rare Disease & Immunology Programme (F1 OPD; F2 Infusion; F3 Ward)
- Precision Oncology — Solid Tumours (F1 OPD; F2 Infusion; F3 Ward; F4 OT; Lower B4 Radiation)
- Genomics, Prevention & Clinical Trials (G Prevention Clinic; F2 Trials Unit; B2 MDT; B3 Genomics Lab)

07 Precision Medicine Centre — Specialist Requirements

The Delhi PMC carries specific infrastructure requirements that are distinct from conventional hospital design and must be addressed from concept stage.

- Precision Prevention Clinic (Ground): designed for well patients — aspirational, wellness-adjacent aesthetic, separated from treatment OPD waiting and urgent care zone
- Genomic Counselling Rooms (F1): soundproofed, minimum 4 rooms, adjacent to waiting with privacy screening; sightlines from nurse station
- MDT Boardrooms (B2): minimum 2, video-linked to VPS Lakeshore Kochi and network tumour boards; adjacent to radiology for direct image review at the screen
- Digital Command Centre (F6): real-time bed / OT / ICU dashboards, AI decision support, telemedicine hub — co-located with BMT to share controlled-access floor infrastructure
- IMP Vault and Cold-Chain Storage (B2): clinical trials investigational product storage, narcotics vault, cleanroom-grade biologic preparation (ISO Class 5)
- Biobank Provision (B3): tissue / DNA / plasma storage, ICMR registry-linkable, adjacent to molecular diagnostics

08 Patient Experience & Environmental Design

- PMC entrance — calm, precision-oriented arrival environment: reflecting clinical intelligence rather than conventional hospital aesthetics; distinct from general hospital entrance language
- Precision Prevention Clinic: aspirational, wellness-adjacent, privately accessed — the design register for well patients making a health investment
- Waiting areas: acoustic control, natural light where possible, dignity-preserving layout — specific attention to the rare disease cohort's long infusion and daycare waiting episodes
- Reduction of visible hospital clutter — clean lines, concealed service infrastructure
- Acoustic performance as a clinical and experiential priority: flooring, ceiling treatment, nurse call systems
- Family communication rooms at each ICU cluster (F5)
- Glass ICUs with privacy controls: clinical observation through glazed partitions, smart glass or privacy screening for dignity
- Neuromuscular zone (F3): extra-wide doors, ceiling hoist-ready design, ventilator bay configuration
- Digitally enabled experience: self check-in, queue management, indoor navigation

09 Infection Control & Safety

- Pressure zoning: positive-pressure BMT (F6), negative-pressure AIIR (F5), negative-pressure isolation rooms (F3 Immunocompromised Ward)
- Dedicated AHU zones at every immunocompromised or high-infection-risk floor — no shared air handling across pressure zones
- Hand hygiene infrastructure at all critical points — clinical entry, bed head, nursing stations
- Biomedical waste flow segregation — dedicated waste lifts and holding areas per floor
- Surface material specification: anti-microbial finishes in ICUs, OTs, BMT suite, and isolation areas
- AIIR suite: 4 rooms minimum, each with anteroom, independent exhaust to atmosphere, negative-pressure monitoring
- Nuclear Medicine Hot Lab: independent ventilation exhaust, negative pressure, no recirculation — Lower B4 location provides structural containment advantage

10 MEP Design & Specification

- HVAC design: OT-grade systems (Class 10,000 / Class 100 as required per room), BMT HEPA H14, AIIR negative-pressure, Transplant ICU anteroom pressurisation
- Medical gas pipeline system design and specification: MGPS, LMO / O_2 plant (Roof), vacuum, AGSS — full building
- Electrical load assessment and redundancy planning: dual power supply, UPS, DG sets; critical care and OT on uninterruptible supply
- Fire alarm and suppression systems — full building including basement levels; special attention to Lower B4 Radiation Oncology vault
- BMS (Building Management System): pressure zone monitoring, temperature, AHU status, bed head panel integration
- CCTV, PA, and Access Control Systems — zoned by programme area; BMT and ICU floors access-controlled independently
- Data network system and WiFi — clinical-grade, RTLS-ready

11 Digital & Smart Hospital Integration

- HIS / EMR integration planning — platform aligned with VPS Lakeshore Kochi network
- Smart nurse call systems
- Real-time patient tracking (RTLS)
- Telemedicine infrastructure — Digital Command Centre (F6) linked to VPS Lakeshore Kochi campus and network tumour boards
- Digital signage and navigation: programme-specific visual identity per clinical vertical
- Real-time bed and OT utilisation dashboards
- Pneumatic Tube System (PTS) — B3 Lab to clinical floors
- AI triage and clinical decision support integration points
- Remote monitoring provision for post-discharge continuity — especially for rare disease and post-BMT patients

12 Phasing & Execution Strategy

The Delhi PMC is a greenfield project. Phasing is accordingly a construction sequencing and commissioning challenge rather than an operational continuity challenge. The radiation oncology basement levels (Lower B4) carry specific sequencing and structural commissioning requirements that must be planned from concept stage.

- Phasing to prioritise: (1) shell and core + MEP rough-in; (2) basement diagnostic fit-out (Lower B4 → B4 → B3 → B2); (3) Ground floor reception and clinical prevention zone; (4) OPD floors (F1); (5) infusion / trials floor (F2); (6) ward floors (F3); (7) OT suite (F4); (8) ICU / HDU (F5); (9) BMT / command (F6)
- LINAC vault 1: commissioned with building opening. LINAC vault 2: structural shell ready from Day 1; commissioning deferred to ~40 Rx patients/day
- Milestone-based execution timeline with clinical team, architects, engineers, and contractors

7. Distinctive Differentiators — Design Catalogue

The following are a few examples of candidate differentiators for evaluation and integration by the design team. Each represents a specific opportunity to distinguish the Delhi PMC through spatial, operational, or technological innovation.

Silent Waiting	Single Visit Diagnosis
Acoustic design and digital queue management in place of audible name-calling. A measurable improvement in the waiting environment — important given the rare disease cohort's long, cyclical visit patterns.	Key clinical pathways resolved within a single visit through co-location of diagnostics and OPD. Especially critical for patients travelling from outside Delhi.

Glass ICUs with Privacy Controls Clinical observation maintained through glazed ICU partitions, with smart glass or privacy screening for patient and family dignity.	**Negative Pressure ICU Pods** All ICUs designed to negative pressure standards as a base condition — pandemic-ready by design. Non-negotiable given the immunocompromised and rare disease patient population.
AI-Assisted Reporting Zones Dedicated workstations for AI-supported radiology and pathology reporting, with infrastructure provision for machine learning integration and genomic data overlay.	**Central Command Centre** Purpose-designed Digital Command Centre (F6) for real-time bed management, patient flow monitoring, OT and ICU dashboards, and telemedicine linkage to the VPS Lakeshore Kochi campus and network tumour boards.
Fast Track Infusion Pathways Rare disease infusion patients (ERT, IVIG, factor concentrates) are a predictable, cyclical cohort. Spatial design to embed fast-track check-in, chair allocation, and discharge to minimise total facility time.	**Family Communication Rooms** Dedicated, enclosed rooms at each ICU cluster for clinical communication with families — privacy, acoustic separation, and dignity by design.

These examples are indicative, not exhaustive and are intended to guide exploration. The design team is encouraged to study international benchmark facilities to understand current state of the art practices and integrate relevant learnings into the design.

8. Concept Design Invitation

Delhi Precision Medicine Centre invites qualified architecture and healthcare design practices to submit a Concept Design Response to this brief.

This is an open concept stage exercise. Submissions will be evaluated on the depth of clinical understanding, originality of spatial thinking, and the coherence of the design proposition in relation to the objectives set out in this document.

Concept Submission Requirements

Responses are expected to address the following areas. Submissions may be in PDF presentation format. Schematic and illustrative representation is appropriate at this stage; detailed construction drawings are not required.

	REQUIREMENT
01 Design Vision	A concise statement of the central design proposition for the Delhi PMC — the governing principle that unifies spatial, clinical, and experiential intent. The response must address how the PMC's quaternary precision medicine identity is expressed architecturally, distinct from conventional hospital design.
02 OT Suite — Sample Layout	Schematic layout of the flagship OT suite (F4) illustrating adjacency relationships: sterile corridor, scrub zone, anaesthesia preparation, recovery bay, and CSSD satellite. Design intent to address material palette, clinical lighting, and HVAC zoning. The transplant OT cluster configuration (3 rooms, perfusion-capable, laminar flow) to be specifically addressed.
03 ICU — Sample Layout	Concept layout for the Haemato-Oncology ICU / HDU floor (F5) — 10 ICU beds, 8 HDU beds, 4 AIIR isolation rooms. Design to address patient visibility, nurse station sightlines, negative-pressure provision, anteroom configuration, family interface, and acoustic performance. The critical separation between F5 AIIR (negative-pressure) and F6 BMT (positive-pressure) to be addressed.
04 PMC Floor — Sample Layout	Illustrative floor plan for one of: (a) the BMT suite (F6), or (b) the Infusion / Day Care / Clinical Trials floor (F2), or (c) the Ground floor Reception / Prevention / Urgent Care zone. Design

	REQUIREMENT
	to address restricted access zoning, pressure differentials (where applicable), material palette, and patient wayfinding.
05 Inpatient Ward — Sample Floor	Illustrative floor plan for the complex care wards (F3) demonstrating bed configurations, nursing station placement, natural light strategy, family zone integration, clean / dirty utility segregation, and isolation room integration. Single, double, and multi-bed room mix to be indicated.
06 Distinguishing Design Elements	A curated set of 3–5 specific design interventions proposed to establish a distinctive identity for the Delhi PMC. At least one intervention to address the precision medicine / genomics identity specifically. At least one to address the spatial and experiential character of the institutional area site context. Precedents, references, and schematic representations may be included.
07 Phasing Strategy	A high-level phasing narrative for a greenfield hospital environment, addressing construction sequencing, commissioning order (infrastructure → diagnostics → OPD → inpatient → critical care), ICRA provisions, and the timeline to first clinical operation.

Evaluation Framework

Submissions will be assessed across five dimensions: (1) Fidelity to brief — depth of engagement with the clinical and operational objectives stated herein. (2) Conceptual rigour — coherence and originality of the design proposition. (3) Clinical credibility — evidence of understanding of hospital operational logic, rare disease programme requirements, and Delhi regulatory context. (4) Spatial intelligence — quality of reasoning demonstrated in sample layouts. (5) Execution viability — plausibility of the phasing and delivery approach for a greenfield facility.

Submission Parameters

- Format: PDF presentation, not exceeding 20 pages, exclusive of practice profile
- Sample layouts at schematic / concept level of development
- Precedent references and illustrative images to be appropriately sourced
- Practice profile to include lead designer, relevant hospital design experience, and completed projects
- All submissions to be treated in strict confidence